

Clinical Diagnosis & History:

Left breast IDC, DCIS, approximately 1.5 cm BRCA2 (+), presents for bilateral TM, left sentinel lymph node biopsy.

Specimens Submitted:

- 1: SP: Sentinel node #1, level 1, left axilla (fs)
- 2: SP: Sentinel node #2, level 1, left axilla (fs)
- 3: SP: Left breast
- 4: SP: Right breast

UUID:63E34973-F630-4295-AC1B-94F818B8BE41

TCGA-AO-A03V-01A-PR

Redacted



DIAGNOSIS:

- 1) LYMPH NODE, SENTINEL #1 LEVEL 1 LEFT AXILLA; EXCISION:
  - ONE BENIGN LYMPH NODE (0/1).
  - DEEPER LEVELS RECUTS AND SPECIAL STAINS HAVE BEEN ORDERED. THE RESULTS WILL BE REPORTED IN AN ADDENDUM.
- 2) LYMPH NODE, SENTINEL #2 LEVEL 1 LEFT AXILLA; EXCISION:
  - ONE BENIGN LYMPH NODE (0/1).
  - DEEPER LEVELS RECUTS AND SPECIAL STAINS HAVE BEEN ORDERED. THE RESULTS WILL BE REPORTED IN AN ADDENDUM.
- 3) BREAST, LEFT; MASTECTOMY:
  - INVASIVE DUCTAL CARCINOMA, NOS TYPE, HISTOLOGIC GRADE III/III (SLIGHT OR NO TUBULE FORMATION), NUCLEAR GRADE II/III (MODERATE VARIATION IN SIZE AND SHAPE), MEASURING 1.5 CM IN LARGEST DIMENSION MICROSCOPICALLY.
  - DUCTAL CARCINOMA IN SITU (DCIS) IS ALSO IDENTIFIED, SOLID TYPE, WITH INTERMEDIATE NUCLEAR GRADE.
  - THE DCIS CONSTITUTES LESS THAN OR EQUAL TO 25% OF THE TOTAL TUMOR MASS, AND IS PRESENT ADMIXED WITH THE INVASIVE COMPONENT.
  - THE INVASIVE CARCINOMA IS LOCATED IN THE UPPER INNER QUADRANT.
  - THE DCIS IS LOCATED IN THE UPPER INNER QUADRANT.
  - NO INVOLVEMENT OF THE NIPPLE BY EITHER IN SITU OR INVASIVE CARCINOMA IS IDENTIFIED.
  - NO CALCIFICATIONS ARE IDENTIFIED IN EITHER THE INVASIVE OR IN SITU COMPONENT.
  - NO VASCULAR INVASION IS NOTED.
  - NO INVOLVEMENT OF THE SURGICAL MARGINS BY EITHER INVASIVE OR IN SITU CARCINOMA IS IDENTIFIED.
  - NO SKIN INVOLVEMENT BY CARCINOMA IS IDENTIFIED.

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ICD-0-3

carcinoma, infiltrating duct, nos 8500/3  
 Site: breast, nos C50.9 lw 10/21/11

| Criteria                       | Yes         | No                                  |
|--------------------------------|-------------|-------------------------------------|
| Diagnosis Discrepancy          |             | <input checked="" type="checkbox"/> |
| Primary Tumor Site Discrepancy |             | <input checked="" type="checkbox"/> |
| IPAA Discrepancy               |             | <input checked="" type="checkbox"/> |
| Prior Malignancy History       |             | <input checked="" type="checkbox"/> |
| Dual/Synchronous Primary Notes |             | <input checked="" type="checkbox"/> |
| Case is (circle):              | QUALIFIED   | DISQUALIFIED                        |
| Reviewer Initials              | lw 10/21/11 |                                     |

- THE NON-NEOPLASTIC BREAST TISSUE IS UNREMARKABLE.
- RESULTS OF SPECIAL STAINS (ER, PR, HER2-NEU) WILL BE REPORTED AS AN ADDENDUM.

- 4) BREAST, RIGHT; MASTECTOMY:
- BENIGN BREAST TISSUE WITH FIBROCYSTIC CHANGES INCLUDING DUCTAL HYPERPLASIA WITHOUT ATYPIA AND APOCRINE METAPLASIA.
  - UNREMARKABLE NIPPLE AND SKIN.

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

\*\*\* Report Electronically Signed Out \*\*\*

Gross Description:

- 1). The specimen is received fresh for frozen section consultation labeled "sentinel node number one, level 1, left axilla" and consists of a single lymph node measuring 2.2 x 1.2 x 1.0 cm, which is bisected and entirely submitted for frozen section consultation.

Summary of sections:  
FSC-frozen section control

- 2). The specimen is received fresh for frozen section consultation labeled "sentinel node number two, level 1, left axilla" and consists of a single lymph node measuring 0.8 x 0.5 x 0.5 cm, which is entirely submitted for frozen section consultation.

Summary of sections:  
FSC-frozen section control

- 3). The specimen is received fresh labeled, "left breast, stitch marks axillary tail, ink margins" and consists of a breast measuring 17.5 x 6.5 x 4.5 cm with overlying skin ellipse measuring 1.5 x 1.4 x 1.1 cm. Situated centrally on the skin surface is an everted nipple measuring 2.2 x 2.0 x 1.0 cm and areola measuring 3.5 x 2.5 cm. The skin is grossly unremarkable. A suture demarcates the axillary aspect. The posterior surface of the breast is inked black and the specimen is serially sectioned to reveal a firm tan-white stellate nodule measuring 1.5 x 1.4 x 1.1 cm, and located 1.2 from the deep margin, in the upper inner quadrant. No additional lesions are identified. Tissue is given to TPS. Sectioning of the axillary aspect

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reveals no grossly identifiable lymph nodes. Representative sections are submitted.

Summary of sections:

N - nipple  
NB - nipple base  
S - skin  
D - deep margin  
T - tumor  
UIQ - upper inner quadrant  
LIQ - lower inner quadrant  
UOQ - upper outer quadrant  
LOQ - lower outer quadrant

4). The specimen is received fresh labeled, "right breast, stitch marks axillary tail, ink margins" and consists of a breast measuring 18.0 x 15.5 x 4.2 cm with overlying skin ellipse measuring 14.2 x 5.2 cm. Situated centrally on the skin surface is an everted nipple measuring 2.0 x 2.0 x 0.7 cm and areola measuring 4.5 x 3.2 cm. The skin is grossly unremarkable. A suture demarcates the axillary aspect. The posterior surface of the breast is inked black and the specimen is serially sectioned to reveal predominant fatty breast tissue with streaks of fibrous tissue. No masses or lesions are identified. Sectioning of the axillary aspect reveals no grossly identifiable lymph nodes. Representative sections are submitted.

Summary of sections:

N - nipple  
NB - nipple base  
S - skin  
UIQ - upper inner quadrant  
LIQ - lower inner quadrant  
UOQ - upper outer quadrant  
LOQ - lower outer quadrant

Summary of Sections:

Part 1: SP: Sentinel node #1, level 1, left axilla (fs)

| Block | Sect. | Site | PCs |
|-------|-------|------|-----|
| 1     |       | fsc  | 1   |

Part 2: SP: Sentinel node #2, level 1, left axilla (fs)

| Block | Sect. | Site | PCs |
|-------|-------|------|-----|
| 1     |       | fsc  | 1   |

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## Part 3: SP: Left breast

| Block | Sect. | Site | PCs |
|-------|-------|------|-----|
| 1     |       | D    | 1   |
| 2     |       | LIQ  | 2   |
| 2     |       | LOQ  | 2   |
| 1     |       | N    | 1   |
| 1     |       | NB   | 1   |
| 1     |       | S    | 1   |
| 1     |       | T    | 1   |
| 2     |       | UIQ  | 2   |
| 2     |       | UOQ  | 2   |

## Part 4: SP: Right breast

| Block | Sect. | Site | PCs |
|-------|-------|------|-----|
| 2     |       | LIQ  | 4   |
| 2     |       | LOQ  | 4   |
| 1     |       | N    | 1   |
| 1     |       | NB   | 1   |
| 1     |       | S    | 1   |
| 2     |       | UIQ  | 4   |
| 2     |       | UOQ  | 4   |

## Procedures/Addenda:

## Addendum

Date Ordered:

Status: Signed Out

Date Complete:

By: --

Date Reported:

## Addendum Diagnosis

## ADDENDUM

## 3). BREAST, LEFT; MASTECTOMY:

Immunohistochemical stains were performed on formalin-fixed tissue with the following results for invasive carcinoma (block T4):

## ESTROGEN RECEPTOR (6F11, +

&gt;95% nuclear staining with moderate to strong intensity

## PROGESTERONE RECEPTOR (1E2; +

&gt;80% nuclear staining with weak to moderate to strong intensity

## HER2 (HercepTest;

Negative (0)

(0% of invasive tumor cells exhibit complete membranous staining;

Uniformity of staining: absent;

Homogeneous, dark circumferential pattern: absent)

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Controls are satisfactory.

Comment: HercepTest™ is an FDA-approved method for assessment of HER2 protein overexpression in breast cancer tissue routinely processed for histological evaluation. The HER2 test results are reported in accordance with the ASCO/CAP guideline recommendations for HER2 testing in breast cancer (J Clin Oncol 2007; 25(1):118-145).

Addendum

Date Ordered: Status: Signed Out  
Date Complete: By:  
Date Reported:

Addendum Diagnosis

ADDENDUM

- 1) LYMPH NODE, SENTINEL #1, LEVEL 1, LEFT AXILLA; EXCISION:
- 2) LYMPH NODE, SENTINEL #2, LEVEL 1, LEFT AXILLA; EXCISION:

- ADDITIONAL H&E STAINED SECTIONS AND IMMUNOHISTOCHEMICAL STAINS FOR CYTOKERATINS (AE1:AE3) SHOW NO EVIDENCE OF METASTATIC TUMOR.

M.D.

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

- 1) FROZEN SECTION DIAGNOSIS: SP: SENTINEL NODE #1, LEVEL 1,  
LEFT AXILLA (FS) : LYMPH NODE, NEGATIVE FOR TUMOR  
PERMANENT DIAGNOSIS: SAME
- 2) FROZEN SECTION DIAGNOSIS: SP: SENTINEL NODE #2, LEVEL 1,  
LEFT AXILLA (FS) : LYMPH NODE, NEGATIVE FOR TUMOR  
PERMANENT DIAGNOSIS: SAME

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Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

- 1) FROZEN SECTION DIAGNOSIS: SP: SENTINEL NODE #1, LEVEL 1,  
LEFT AXILLA (FS) : LYMPH NODE, NEGATIVE FOR TUMOR  
PERMANENT DIAGNOSIS: SAME
- 2) FROZEN SECTION DIAGNOSIS: SP: SENTINEL NODE #2, LEVEL 1,  
LEFT AXILLA (FS) : LYMPH NODE, NEGATIVE FOR TUMOR  
PERMANENT DIAGNOSIS: SAME

\*\* End of Report \*\*